



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : صفاء زكي قاسم حميده

العنوان : الجيزه

الحي / المدينة : اخري

المحافظة : الغربية

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 25810101601966 رقم التليفون : 01007705873

العمر (تاريخ الميلاد) : 10-10-1958 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج كيماوي تاكسان (دوسيتاكل) قسم العلاج : الاورام [14152]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: صفاء زكي قاسم حميده National ID: 25810101601966	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Docetaxel chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

صفاء

زكى قاسم حميده

٢١ ش هلال - ش عز الدين عمر - اسبائس

العمرانية - الجيزة



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IR2882629

DMS
فيسجو



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ربة منزل



متزوجة

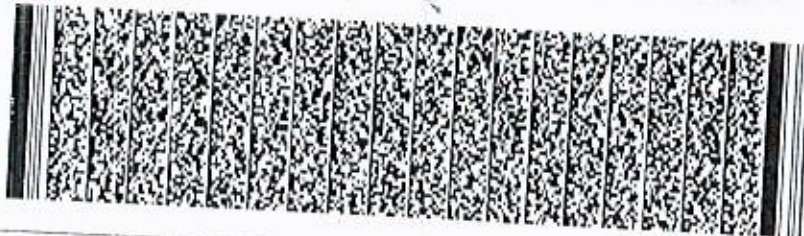
مسلمة

أنثى

عبد السلام حسن عطيه محمد

البطاقة سارية حتى ٢٠٢٧/١١/٢٩

١



MDT FORM

Name:	سعاد زكي لاسم حبيب	MRN:	8676
Age:	68 Y	ID:	25810101601266
Menopausal Sta:	POST	Phone:	01032084198
NRS SCORE:		Marital Status:	
BMI:		Family History:	+VE SISTER BREAST CANCER
CO Morbidity:	DM ORAL CONTROLLED, HTN	Reason Of MDT:	REHOT

Clinical Data:

BX: RT. BREAST IDC GIII
ER: 8/8 PR: 7/8 HER2: +3 KI67: 1

R

L

Sonomammography:

CESM RT> UPPER CENTRAL MASS 6.3*3.5*2.3CM 8CM (INDEX LESION)
UIQ RETOAREOLAR MASS 1*1.3*0.5CM 7CM FROM INDEX LESION 0.4 CM
FROM NIPPLE
OVER ALL AREA 13*3.5*2.3CM
NO PATHOLOGICAL LN BIRADS 6
LT.5 OCLOCK MIDDLE THIRD FOCAL DUCT 5MM WITH SMALL NODULE 5MM
LOCATED 4 CM FROM NIPPLE

TRU CUT LT BREAST MASS >>. INTRDUCT PAPILLOMA NO MALIGNANCY

Meta Workup:

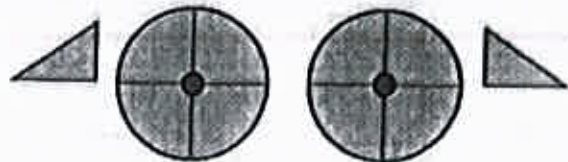
BONE SCAN >>> FREE
INC SAME RESULT AFTER REVISION

History:

Recommendations:

MDT Outcome & Plan

CLIP Insertion in the 2 lesions of rt breast
1/ PORTA CATH INSERTION
2/NACTH CTH TCHP PROTOCOL *18W + ECHO



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ حنان حوت	د/ احمد نجيب				

Patient Name	: الأستاذ / صفاء زكي قاسم حميدة	Request No.	: 13004977
Patient No.	: 25810101601966	Request Date	: 10/02/2026
Path No.	: SR-224/26	Verification Date	: 19/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 67 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast 12 O'clock mass.
Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

One referred block labelled by (210-26). Together with, four referred slides including 1 H&E stained slide and 3 immunostained slides for (ER, PR, HER2neu).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * DCIS: Focal high grade ductal carcinoma in situ (Solid pattern with comedo type necrosis).
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.

* IPT:

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 50% of tumor cells (score 6/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.
- Focal high grade ductal carcinoma in situ (Solid pattern with comedo type necrosis).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 6/8).
- Her2: Positive (score 3+).

Verified by :

Dr.Duaa Mohamed

Printing Date : 18/03/2026 12:19:31

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / صفاء زكي قاسم حميدة	Request No.	: 13004977
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Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 67 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

٢٦٧٦

Name : Safaa Zaki Qassem Hamida

Branch : Dokki

Ref.Physician Prof : Goustav Roussey Egypt

Date : 09-04-2026 01:41

Medical No. : 346739

Ins.Company : تعاقد مستشفيات

Tc-99m MDP ISOTOPIC BONE SCAN

Clinical History: Female, 65 years old; history of breast carcinoma. Evaluation for possible skeletal metastases.

Technique: Whole-body planar bone scintigraphy was performed in anterior and posterior projections following appropriate uptake time. Additional spot views were obtained when necessary.

Findings:

- The skeletal system demonstrates physiological radiotracer distribution with normal uptake in the axial and appendicular skeleton.
- No focal abnormal areas of increased or decreased tracer uptake are identified to suggest skeletal metastatic deposits on scintigraphic basis.
- Expected age-related degenerative changes may be seen (at different levels of the spine and around big joints) without suspicious metastatic pattern.

IMPRESSION:

- No scintigraphic evidence of skeletal metastases on whole-body bone scan.
- Follow-up imaging only if clinically indicated.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Patient Name : الأستاذ / صفاء زكي قاسم حميده

Patient No. : 25810101601966

Path No. : 935/26

Sex : Female

Age : 67 Y

Referring Doc :

Request No. : 13005774

Request Date : 08/04/2026

Verification Date : 14/04/2026

Financial Category : جوماتاف روسي - نفقة الدولة

Department : Out Patient

Registered by : .marwa mohamed youss

Pathology Report

CLINICAL DATA

- Right breast pattern proved malignant. (BIRADS 6).
- Left breast small mass (BIRADS 3)

NATURE OF SPECIMEN

- Left breast mass, tru cut biopsy taken at 9:35 p.m.

GROSS EXAMINATION

- Fragmented grey white tissue cores ranging in size from 0.2 cm up to 1.5 cm, totally submitted.

MICROSCOPIC EXAMINATION

Microscopic examination revealed normal breast tissue showing multiple circumscribed intraductal proliferation comprised of arborizing fibrovascular cores lined by outer layer of luminal cells and an inner layer of myoepithelial cells. Also there was fibrocystic changes , adenosis , apocrine metaplasia , microcalcification & focal ductal hyperplasia . No malignancy.

DIAGNOSIS

- Left breast mass, TCNB:
- Intraduct papilloma. No malignancy .

Signature

Dr. Walaa Ibrahim Ghoneim

Dr.Heba Hassan Adly

Verified by :

Dr Maha Guimie

Printing Date : 19/04/2026 10:45:14

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Patient Name	: الأستاذ / صفاء زكي قاسم حميده	Request No.	: 13005774
Patient No.	: 25810101601966	Request Date	: 08/04/2026
Path No.	: 935/26	Verification Date	: 14/04/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 67 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Verified by :
Dr Maha Guimie

Printing Date : 19/04/2026 10:45:14

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Name: Safaa Zaki Kazem

Date: 31-03-2026

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: known case of right IDC III with focal high grade insitu component.

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: B (scattered fibro-glandular densities)

LEFT BREAST:

- Low energy (Non contrast) images: outer central middle third focal asymmetry.
- No spiculated masses, or distortion or suspicious microcalcifications.
- Recombined (Post contrast) images: outer central small mild enhancing focus measures 5mm located 4cm from nipple.
- Normal skin / nipple / areolar complex.
- Complementary US: 5:00 middle third focal duct dilation up to 5mm with small circumscribed hypoechoic nodule that could be roughly correlated to the contrast findings.

RIGHT BREAST:

- Low energy (Non contrast) images :
 1. Upper central posterior third irregular mass with indistinct margins and mixed density.
 2. UIQ retroareolar focal asymmetry with faint calcific foci
- Recombined (Post contrast) images
 1. Upper central posterior third heterogeneous intense enhancing irregular mass lesion with superior non enhancing cystic component, it measures 6.3x3.5x2.3cm located 8cm from nipple.(index lesion)
 2. UIQ retroareolar indistinct moderate heterogeneously enhancing lesion measures 1x1.3x0.5cm 7cm away from the index lesion and 0.4cm from nipple correspond to the low energy image
 3. The overall coverage distance 13x3.5x2.3cm
- Complementary ultrasound:
 1. 12:00 irregular partially solid partially cystic lesion(index lesion)
 2. 1:00 retroareolar dilated thick wall ducts with calcific foci seen within.
 3. No pathological axillary lymphnodes

IMPRESSION:

LEFT BREAST: (BIRADS 3= probably benign): outer central enhancing focus as described , probably benign for US guided biopsy

RIGHT BREAST: (BIRADS 6= pathologically proved malignancy) upper central posterior third & UIQ retroareolar two pathological enhancing lesion as described

Best regards,

Dr. Marwa Ramzy, MD, FRCR

Name : safaa zaki kasem

Branch : Portable

Ref.Physician Prof : ct cap+c

Date : 01-03-2026 02:52

Medical No. : GRI-25810101601966

Ins.Company :

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

****Follow up of right breast cancer:**

****No available previous imagings:**

CECT CHEST:

- Right breast enhancing soft tissue mass lesion measures 33 x 27 mm.
- Multiple bilateral subcentimetric axillary lymph nodes at level I.
- No cavitary lesions or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

CECT ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous CT density with no enhancing focal lesion or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows tiny dense stone.
- Diffuse circumferential smooth gastric wall thickening reaching 26 mm.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.

THE SCANNED SKELETON:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-ossseous soft tissue component.

OPINION:

- Right breast enhancing soft tissue mass lesion.
- Multiple bilateral subcentimetric axillary lymph nodes at level I.
- Diffuse circumferential smooth gastric wall thickening ...For clinical correlation.
- Calcular gall bladder.
- No pulmonary nodules.
- No hepatic lesions could be detected.

Dr. Heba El Balshy, M.D.
Lecturer and Consultant of Diagnostic and Interventional Radiology
M.I. Menoufia University

BaHEYA

PATIENT NAME: SAFAA ZAKY KASEM HAMEDA

GENDER: Female

DATE : 13-Nov-2025 10:30 AM

PATIENT ID: 10125273997

AGE: 67 y

ORDER: 3641508,3641509

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Heterogeneously dense breast parenchyma (ACR C) with scattered calcific foci.
- A partially circumscribed dense lesion with surrounding focal asymmetry is seen in the right UOQ; complementary ultrasound revealed an 12:00 intercommunicating lesions giving rise to an overall irregular lesion of mixed echogenic nature including anechoic, hyperechoic (layered content) and hypoechoic components for a collective measurement of 27x14x28mm (respective TSXAPXCC diameters). There is associated vascularity on application of color Doppler.
- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no other solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

Right breast: adenosis and benign calcifications with a complex cystic lesion, BIRADS 4a for biopsy.

Left breast: adenosis and benign calcifications, BIRADS 2.

RECOMMENDATION:

Reported By,

Dr. SHEREHAN WAHEED, MD

13-Nov-2025 10:59 AM

PATIENT: SAFAA ZAKY KASEM HAMEDA, 10125273997, Female, 67 y
Report version 1, 13-Nov-2025 10:59 AM

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16802



[[DUPLICATE MAMMOGRAM]]



Patient's Name: Safaa Zaki

Date: 2026-09-10

Referred by: Prof. Dr. no doctor

Hospital: Goustave Roussey International Egypt

ULTRASOUND & COLOR DOPPLER REPORT (PORT A CATH):

Left Internal Jugular Vein (LIJ):

- Fully patent, demonstrating normal complete luminal compressibility and smooth thin walls.
- No evidence of mural thickening or intraluminal thrombosis.
- Color Doppler and spectral analysis reveal no obstruction.

Port Reservoir & Pocket:

- The subcutaneous reservoir is identified in the left chest wall with expected posterior acoustic shadowing.
- There is mild subcutaneous tissue echogenic fat with no localized collection.
- No surrounding fluid collections or localized hematoma at the port pocket.

Catheter Track & Course:

- The catheter is identified entering and traversing the lumen of the lower left internal jugular vein toward the jugular-subclavian junction.
- Visualized segments of the catheter appear intraluminal with no visible surrounding thrombus or gross echogenic cuffing along its accessible cervical course.

Conclusion:

- Completely patent left internal jugular vein with normal Doppler parameters and no evidence of deep venous thrombosis (DVT).
- The intraluminal course of the visualized cervical segment of the right port a catheter with no detectable peri-catheter thrombosis.
- Mild inflammatory changes at the skin overlying the port a cath, as described.

Best regards;
Dr. Mohammad Islam

GUSTAVE /
ROUSSY /
INTERNATIONAL
HOSPITALدوستانی
روسی
انترنیشنل
ہسپتالECHO REPORT

NAME:

Paulo José

DATE: 10 / 9 / 2026

ID NUMBER:

/

SEX: ♀ AGE: 65

EF	63%	55-75%	PWT	1.1	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVEDS		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

normal

LV Wall Thickness

normal

LV Diastolic Function

DD grade I

SWMA

normal

LA and Ao Root Dimensions

normal

Mitral Valve

normal

Aortic Valve

Sclerotic AV with no sig. systolic gradient

Tricuspid Valve

normal

RUSP 29mmHg

Pulmonary Valve

normal

Right Side and Systolic Function

normal

Intracardiac Masses and Effusion

normal

DR

Jesús